

Therapy Frequency Justification Guide

How to support your frequency · What CMS looks for · Documentation language · 2026

Therapy frequency must be clinically justified for every patient at every frequency level. "We always do 5x/week" is not a clinical rationale. Frequency should be driven by the patient's acuity, rehabilitation potential, and response to treatment. CMS auditors specifically look for individualized frequency justification — not blanket protocols.

Frequency Levels — When Each Is Appropriate

	Clinically appropriate when:	Documentation must include:
5x / week	• Acute post-surgical (hip, knee, spine) within first 1–2 weeks • Acute neurological event (CVA, TBI) in early recovery phase • Significant functional decline requiring intensive daily intervention • Patient has high rehab potential and can tolerate daily treatment • Complex multi-system presentation requiring daily reassessment	• Specific reason daily treatment is medically necessary • Why less frequent treatment would compromise outcomes • Patient's tolerance and response to daily intervention • Expected timeline for stepping down frequency
3–4x / week	• Post-acute phase — patient making steady progress but not requiring daily skilled care • Moderate functional deficits with good rehab potential • Patient fatigues with daily treatment or has medical constraints • Appropriate for most standard SNF patients after initial acute phase • Maintenance phase with skilled monitoring needs	• Clinical rationale for this specific frequency vs daily • Patient's response to treatment and tolerance level • Progress markers that will guide frequency adjustment • Any medical or endurance factors limiting higher frequency
1–2x / week	• Maintenance program requiring skilled monitoring only • Patient nearing discharge — winding down skilled need • Low complexity presentation with primarily HEP carryover • Patient preference or medical constraints limit frequency • Restorative transition phase	• Clear clinical rationale for reduced frequency • What skilled monitoring or oversight still requires a therapist • Discharge timeline and plan • How HEP or restorative program bridges between visits

Weak vs Strong — Frequency Justification Language

✗ Weak Frequency Justification	✓ Strong Frequency Justification
<i>"Patient to be seen 5x/week per facility protocol."</i>	<i>"Daily skilled PT is medically necessary secondary to acute post-hip fracture status, WBAT precautions, and complex transfer training needs. Patient requires daily reassessment of WB tolerance and technique modification."</i>
<i>"Patient will be seen 3x/week for strengthening."</i>	<i>"3x/week frequency is appropriate at this phase — patient has completed intensive daily phase and is now demonstrating carryover of skilled instruction. Less frequent treatment allows active integration of HEP between sessions."</i>
<i>"Frequency reduced from 5x to 3x per MD order."</i>	<i>"Frequency stepped down from 5x to 3x/week as patient has progressed through the acute post-surgical phase. Current presentation supports 3x/week with HEP for active days. Patient tolerating well with no setbacks."</i>
<i>"Patient seen 2x/week for maintenance."</i>	<i>"Skilled PT 2x/week is required to monitor patient's maintenance program and assess for early signs of decline. Non-skilled staff cannot safely modify or monitor this program per Jimmo standard."</i>

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Stepping Frequency Up or Down — Documentation Requirements

Stepping UP (e.g. 3x → 5x)

Document the clinical trigger: new fall, functional decline, change in medical status, new surgical procedure, or response to treatment indicating more intensive intervention is needed. State specifically why the current frequency is no longer sufficient and what daily treatment will address.

Stepping DOWN (e.g. 5x → 3x)

Document the clinical rationale: patient has progressed through the acute phase, is demonstrating carryover, can integrate HEP between sessions, or has medical/tolerance limitations. Do not just reduce frequency — document why the lower frequency is now clinically appropriate.

Holding Frequency (no change)

If frequency stays the same despite a plateau, document why the current frequency is still necessary. Per Jimmo — continued skilled care at current frequency is appropriate if skilled monitoring or intervention is still required to maintain function or prevent decline.

Audit red flag: All patients on your caseload at the same frequency is a major audit trigger. Frequency must vary based on individual patient need. A building where every patient is seen 5x/week signals that frequency is being driven by productivity targets, not clinical need.

Payer-Specific Frequency Guidelines

Medicare Part A (PDPM)

Under PDPM (effective Oct 2019), CMS does NOT mandate a minimum number of therapy minutes or a required frequency. Therapy minutes no longer drive payment tiers as they did under RUG-IV.

Standard practice: 5x/week across disciplines (PT, OT, ST) during the acute phase of a Part A stay is clinically appropriate and supports optimal Section GG functional outcomes and patient recovery. However, frequency must still be individually justified based on the patient's clinical presentation — not scheduled uniformly for all Part A patients.

Key: Section GG admission and discharge scores drive PT/OT payment under PDPM. Accurate scoring and meaningful functional improvement documented in therapy notes support both clinical quality and appropriate reimbursement.

Managed Care / Medicare Advantage

Managed care plans (Medicare Advantage, HMO, PPO, and commercial) each have their own contracts with specific requirements for therapy frequency, authorized minutes, visit limits, and prior authorization rules.

Always follow your specific contract terms and authorized minutes for each managed care payer. Do not assume that what is appropriate for Medicare Part B applies to managed care — requirements vary significantly by plan.

Key actions: (1) Verify authorization before starting treatment. (2) Track authorized visits and minutes — do not exceed without re-authorization. (3) Submit progress notes per the plan's requirements to support continued authorization. (4) Contact your billing team if a plan denies or reduces authorized frequency.

Medicare Part B

Frequency is driven entirely by clinical need and skilled justification — see the frequency levels guide above. There are no CMS-mandated minimum or maximum visit frequencies for Part B therapy. The only standard is: frequency must be medically necessary, individually justified, and documented with skilled need for each session.

Key: Part B is billed fee-for-service. Every visit must stand alone as clinically justified. Do not maintain a frequency that exceeds what the patient's clinical presentation supports.